AI FOR GOOD NIGERIA SUMMIT 2026 · ABUJA · 10 DECEMBER

# Nobody in the room speaks both languages.

Unmute is the two-way interpreter that fits in a nurse's pocket.

**Prince Nzubechukwu Michael · Founder, Unmute**

contact@liveunmute.com · liveunmute.com · app.liveunmute.com

THE WARD, TODAY

# The interpreter is not in the room.

1. An NSL interpreter has to be booked. In practice that means days — and often an interpreter in another state.

2. So the encounter is carried by a relative, or by a pen and a scrap of paper.

3. The three things that must never be guessed — where it hurts, what the patient is allergic to, and whether they consent — are exactly the three that get guessed.

4. The law already requires access: the Discrimination Against Persons with Disabilities (Prohibition) Act 2018.

THE MEASUREMENT

Nigeria publishes no ratio of Deaf patients to qualified interpreters. The gap is not measured, which is part of why it does not move.

THE PRODUCT

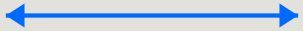
# Two directions. One screen.

### PATIENT → CLINICIAN

The patient signs into the camera.

Hand shapes are read on the device.

English appears on the clinician's screen, and is spoken aloud.



### CLINICIAN → PATIENT

The clinician speaks.

Speech becomes text in the browser.

The reply is played back to the patient in sign.

Runs in the browser of the phone the nurse already carries. Nothing to install. No account. Works on ward wifi after the first load.

STATUS

# What works today. What does not.

## EXISTS

— Live camera capture and hand tracking, on the device

— Reads yes, no, not sure, and the numerals 1 to 5

— Clinician speech to text; the patient's answer spoken aloud

— Refuses out loud when a phrase falls outside its set

— Runs in a browser, on ward wifi

## DOES NOT EXIST

— English to signed video — a placeholder today, twelve clips in production

— A trained NSL model

— A published accuracy number

— A hospital pilot

— A Deaf patient who has used it

— Revenue

**The refusal is the feature. A tool that invents a dosage is worse than one that admits it did not understand.**

THE WEDGE

# Two hundred clinical signs, not a bigger model.

### WHAT WE LICENCE

Translation between English and sign, both directions. Bought, not built.

### WHAT WE OWN

The clinical vocabulary. The refusal behaviour. The record of what was understood, and what was not.

---

**200** signs and phrases that carry most of the routine clinical risk — pain location, severity, onset, allergies, medication, consent.

**1** confirmation step: on anything critical, the patient approves what was understood before it reaches the clinician.

**0** guesses. Outside the vocabulary the app says so, and asks a human to step in.

General sign models are trained on broadcast and lecture footage. They know family, school and weather. They have never seen hypertension, NPO, or milligrams.

THE ASK

# What we want from this summit.

**01** **Two pilot departments**

One general hospital and one community clinic in the FCT. Free for six months. In exchange: ward access, recorded sessions with patient consent, and the vocabulary that actually gets used.

**02** **NSL advisers, paid**

Deaf advisers and qualified interpreters to build the two hundred terms. Not consulted once — employed.

**03** **Introductions**

NITDA and Federal Ministry of Health digital health programmes. State hospitals already carry accessibility obligations.

Funding: self-funded. A pre-seed round is open, but the pilot matters more than the cheque.

**Prince Nzubechukwu Michael · Founder, Unmute · contact@liveunmute.com · liveunmute.com**